

DEFENSE COMMERCIAL SOLUTIONS OPENING (CSO)

AREA OF INTEREST (AOI)

Periodic Health Assessment (PHA) Modernization Capability

In Support of the Profiling and Readiness Action Tracking Unified System (PARATUS)

1. Authority and Purpose

This Area of Interest (AOI) solicits innovative commercial solutions, or new applications of existing commercial technology, to close a defined capability gap in the Periodic Health Assessment (PHA) process governed by DOWI 6200.06 (Periodic Health Assessment Program) and DOWI 6025.19 (Individual Medical Readiness).

This AOI does not constitute a Statement of Work or Performance Work Statement. It describes the Government's area of interest and the technical data and evaluation factors that will govern proposal selection. Offerors retain latitude to propose the technical approach, architecture, and implementation method that best satisfies the stated capability need, including proposing as part of a team consistent with Section 4.6.

Following successful completion of Phase 2 (CDP Conformance Gate), the Government may, in its discretion, award a prototype Other Transaction agreement under 10 U.S.C. § 4022 to one or more offerors or teams who have passed Phase 2, using the competitive procedures conducted under this CSO through Phase 2 to satisfy the competitive-procedures prerequisite of 10 U.S.C. § 4022. Award of an OT agreement under this provision does not constitute completion of this AOI's technical evaluation; Phases 3 through 6 continue as post-award milestones under that agreement, administered per Section 4.9. Upon an offeror's or team's successful completion of Phase 6, whether reached through the standard AOI down-select described at Sections 4.7 and 4.8 or through post-award milestones under a prototype OT agreement awarded under Section 4.9, the Government may, in its discretion, either (a) extend or convert that offeror's or team's prototype OT agreement to a follow-on production agreement which may include a sole-source follow-on production contract under 10 U.S.C. § 4022. Nothing in this section commits the Government to any award, or alters the Government's discretion described at Section 6.

2. Background and Capability Gap

The PHA is the annual, comprehensive medical readiness assessment required of Active and Reserve Component Service members across the Military Services, documented on DD Form 3024. The current PHA is completed in four sequential parts: Part A (Service Member self-assessment), Part B (Record Review, completed by a designated Record Reviewer), Part C1 (Mental Health Assessment, completed by an MHA Provider), and Part C2 (PHA Health Care Provider review, recommendations, referrals, and certification). There are current policy-focused efforts to reform the PHA, by shortening unnecessary questions and aligning the timing to replace other joint health assessments.

No automated, AI-enabled, or risk-prioritized capability currently exists to support completion of the PHA utilizing a common data framework and hosting architecture. The present-day process — spanning Military Service Readiness Systems and Service-specific PHA portals — is manual and forms-driven: Service members complete static web-based questionnaires; Record

Reviewers manually cross-check immunization, laboratory, and deployment documentation; MHA Providers and PHA Health Care Providers (collectively Clinical Readiness Staff, along with Record Reviewers) manually review structured and free-text responses without machine-assisted triage; and unit commanders receive completion and readiness status through static, periodically generated reports rather than real-time notification. Service members in remote or geographically dispersed locations — including a significant share of Reserve Component personnel — face additional access barriers because existing tools are largely desktop/web-only and not designed for mobile completion.

Closing this gap also requires that PHA data be curated, governed, and made available within the common data backbone supporting PARATUS — not just modernized at the point of capture. This AOI accordingly addresses the following capability gap: a streamlined, AI-enabled, risk-prioritized, multi-platform PHA capability. Every capability described in Sections 4.2 and 4.3 represents a gap to be closed through commercial competition, not an existing system to be extended, and is intended to inform the broader PARATUS consolidation of Military Service medical readiness systems.

3. Statement of Government Interest

The Government is interested in commercial solutions — software, software-as-a-service, or a new application of an existing commercial product — that modernize completion of the PHA (DD Form 3024, Parts A, B, C1, and C2) through raw data to decision-ready data, agentic AI, machine learning (ML)-based risk scoring, automated notifications, and visualization dashboards, accessible via both web and native or responsive mobile platforms, suitable for operation within a DOW IL5 environment, capable of DOW IL6 cross domain solution (CDS) connection and capable of integrating with Military Health System data sources (e.g., MHS GENESIS, FHIR R4, HL7, structured and unstructured).

Solutions of interest should demonstrate, at minimum:

- Agentic AI that guides Service members through Part A self-assessment via adaptive, conversational interaction rather than static questionnaire logic, while preserving the full DD Form 3024 data set required for Individual Medical Readiness (IMR) determination.
- AI- or ML-based risk scoring that triages completed assessments for Clinical Readiness Staff attention, prioritizing review queues by clinical and readiness risk rather than strict first-in-first-out order.
- Automated notification and reminder capability spanning Service members, Clinical Readiness Staff, and unit commanders, tied to PHA due dates, overdue status, risk changes and IMR category changes.
- Visualization dashboards providing real-time, part-level PHA completion and risk status at the individual, unit, and command level.
- Native and/or responsive mobile accessibility, in addition to full web access, with CAC/PKI or equivalent DOW-approved authentication.
- Demonstrated or readily achievable integration with authoritative Military Health System data sources. IL5 hosting/data governance constraints, and IL6 CDS.

- Conformance with the Government-furnished Contract Data Package described at Section 4.5, sufficient to ingest, transform, and govern PHA-domain data within the common joint-service medical readiness data backbone, without dependence on access to a Government-hosted environment.
- Demonstrated ability to connect to, and exchange data with, the Government-provisioned test environment described at Section 4.5.4, using either native deployment within that environment or a secure remote connection, consistent with the offeror's proposed hosting approach at Section 4.9.
- Human-in-the-loop safeguards ensuring AI- and ML-generated outputs function strictly as clinical and administrative decision and process support, with all diagnostic, referral, and readiness determinations retained by a licensed Health Care Provider or other designated official.

4. PHA Capability Requirements — Down-Select Discriminators

The PHA capability described in Section 4.2 and the data validation described in Section 4.5 are the primary technical discriminators for down-select under this CSO. Proposals will not advance from phase to phase, as described in Section 4.7, on the basis of price, narrative quality, general technical merit, or prior corporate experience with DOW/DHA-specific data environments alone.

4.1 Intent

The PHA capability requirement reflects the Government's intent to replace a disconnected, data-deficient, static, manual, forms-driven assessment process with a streamlined, AI-enabled, risk-prioritized capability spanning all four parts of DD Form 3024. The data validation requirement reflects the Government's parallel intent to establish, within the program's 8-month credibility window described at Section 6, a working integration of PHA data into the common joint-service data backbone that subsequent Areas of Interest under this CSO will build upon. The capability is intended to reduce administrative burden on Service members and Clinical Readiness Staff, accelerate identification and routing of elevated-risk cases, and provide commanders and program offices with real-time, decision-ready medical readiness status — consistent with the Defense Health Agency's medical readiness modernization objectives and the broader PARATUS effort to unify joint-service medical readiness data.

The Government's intent in defining the validation requirement at Section 4.5 is to evaluate an offeror's or team's ability to conform to a Government-defined data and to connect that conformant data into a Government-provisioned environment, not to require that an offeror have independently built an enterprise-scale orchestration backbone prior to competing. This distinction is deliberate: it is intended to keep this AOI open to offerors with mature PHA capability who may not have prior experience specifically within the DOW or DHA data environment, whether competing individually or as part of a team under Section 4.6.

This AOI's data validation requirement comprises two distinct, sequentially gated elements: conformance to the published Contract Data Package (Section 4.5.1), and live connectivity to the Government-provisioned test environment (Section 4.5.4). These are evaluated as separate gates because they test materially different capabilities — interface conformance versus

operational connectivity — and because collapsing them into a single gate risks a failure in one being masked by success in the other.

4.2 Required Capabilities by PHA Part

Offerors must demonstrate each Part below in full during the Phase 1 capability demonstration described in Section 4.7. Within each Part, the cross-cutting capabilities at Section 4.4 — agentic AI, ML risk scoring, notifications, visualization dashboards, and web/mobile accessibility — apply as described.

Part A — Service Member Self-Assessment

- Agentic, conversational intake that dynamically branches questioning based on prior responses (e.g., deployment history, occupational exposures, women's health, behavioral health screening items), consistent with the content areas of DD Form 3024, Part A.
- Automated ingestion and pre-population of Service Member's problem list, allergies, active medications, procedure history, immunization, laboratory, deployment, and medical readiness profile data from authoritative sources, to reduce manual Record Reviewer lookups.
- Preliminary AI or ML risk flagging at the point of self-assessment, to pre-stage likely referral or follow-up needs for downstream reviewers.
- Automated push notifications and reminders for upcoming, due, overdue PHA requirements, and changes in risk profile delivered to the Service member via web and mobile channels.
- Full completion parity between mobile and web platforms; no element of Part A may be restricted to desktop-only access.

Part B — Record Review

- Automated ingestion and pre-population of Service Member's problem list, allergies, active medications, procedure history, immunization, laboratory, deployment, and medical readiness profile data from authoritative sources, to reduce manual Record Reviewer lookups.
- AI- or ML-based completeness and discrepancy scoring that flags missing or inconsistent documentation prior to Record Reviewer assignment.
- Risk- and suspense-date-based queue prioritization and notification to Record Reviewers.
- Dashboard visualization of Record Review throughput, backlog, and aging by unit and organization.

Part C1 — Mental Health Assessment (MHA)

- AI- or ML-based risk scoring of MHA responses to support, and not replace, MHA Provider triage and prioritization of follow-up.
- AI- or ML-based summarization of Service member self-reported behavioral health responses to reduce MHA Provider review time, with the complete underlying responses always available to the Provider in full.

- Automated, secure notification to MHA Providers of pending and risk-prioritized assessments.
- Mandatory human-in-the-loop control: AI- or ML-based risk scores in Part C1 shall be advisory only. Behavioral health determinations remain the sole responsibility of the licensed MHA Provider, and the proposed solution must make this division of responsibility explicit in its workflow design.

Part C2 — PHA Health Care Provider Review, Recommendations, and Referrals

- Composite, multi-part risk visualization — integrating Part A, Part B, and Part C1 inputs — presented to the PHA Health Care Provider in a single dashboard view prior to certification.
- Agentic AI-assisted drafting of recommendation, referral language, and/ or Deployment Limiting Medical Conditions language based on flagged risk items, subject to mandatory Provider review, edit, and digital signature/certification before any recommendation or referral is finalized.
- Automated notification to unit commanders and Service members of Individual Medical Readiness (IMR) category determination (Fully Medically Ready, Partially Medically Ready, Not Medically Ready) immediately upon Provider certification.
- A persistent audit trail distinguishing AI/ML-suggested content from Provider-entered or Provider-approved content, to preserve clinical accountability and support after-action review.

Additional Offeror-Proposed Capabilities & Modernization Transition

The requirements in Section 4.2 represent the Government's minimum baseline capability interest to satisfy the Phase 1.b software demonstration; they are the floor, not the ceiling. Because the Government's ultimate intent is to move beyond the current forms-based paradigm, offerors are expected to propose and showcase advanced capabilities that are not explicitly enumerated above.

Offerors must use the Phase 1.a Strategic Vision presentation (detailed in Section 4.3) to demonstrate these forward-looking capabilities—such as continuous health monitoring, non-traditional data integration, or proactive risk identification. Failure to propose a capability that extends beyond the baseline digitization of the current PHA process will result in an unfavorable qualitative evaluation at Phase 1.a gate.

Any additional capability demonstrated remains subject to the human-in-the-loop safeguard in Sections 3 and 4.4: no additional capability may substitute machine determination for licensed Health Care Provider judgment on a clinical or diagnostic matter.

4.3 Strategic Vision and Complexity Understanding (The "Art of the Possible")

The Government's intent is not merely to digitize the existing DD Form 3024, but to transition to a continuous, data-driven medical readiness profile where static forms become an archive-only artifact. Offerors must demonstrate a clear strategic vision for how their architecture, data orchestration, and approach will support this future state within the broader PARATUS ecosystem.

As a primary qualitative gate, offerors must articulate:

- **A Post-Forms Future:** How their solution transitions the PHA from a static, episodic data-collection event to a dynamic, continuous profiling capability.
- **Practical Innovation:** A practical use-case of applying agentic AI and continuous data feeds (e.g., wearables, disparate health records, non-traditional data sources) to proactively identify readiness risks outside the annual PHA cycle.
- **Complexity Deconstruction:** A critical deconstruction of the current process, demonstrating a deep understanding of the clinical, operational, and data complexities required to modernize the DOW/DHA medical readiness ecosystem, rather than simply presenting a generic technology platform.

4.4 Cross-Cutting Technical Requirements

The following capabilities apply across all four Parts of the PHA and are evaluated holistically, not only within a single Part:

- **Agentic AI** — adaptive, task-completing AI behavior embedded in intake, review, and recommendation workflows, as distinct from static rules-based form logic.
- **AI- or ML-Based Risk Scoring** — a documented, defensible methodology for deriving risk indicators from PHA data, with demonstrated accuracy and bias-testing approach.
- **Notifications** — automated, role-aware notification routing across Service members, reviewers, providers, and commanders.
- **Visualization Dashboards** — real-time, role-appropriate dashboards at the individual, unit, and command level.
- **Web and Mobile Accessibility** — full functional parity across web and native/responsive mobile platforms, with DOW-approved authentication.
- **IL5 Hosting w/ IL6 CDS and MHS Data Integration** — demonstrated or readily achievable compatibility with IL5 hosting, IL6 CDS and integration with authoritative Military Health System data sources (e.g., MHS Genesis FHIR R4), conformance with the Contract Data Package described at Section 4.5.1, and connectivity to the Government test environment described at Section 4.5.4.
- **Human-in-the-Loop Safeguards** — explicit preservation of licensed Health Care Provider authority, or Independent Duty Enlisted Provider with any necessary oversight, over all clinical and diagnostic determinations; AI/ML outputs are decision support only.

4.5 Data Validation Requirement

4.5.1 Government-Defined Integration Target: the Contract Data Package

The Government will furnish, in connection with the Phase 2 down-select gate described at Section 4.6, a Contract Data Package consisting of: (1) the target schema and data dictionary for PHA-domain data elements, consistent with the medallion-pattern (raw/conformed/curated) structure used across PARATUS; (2) FHIR R4 profile constraints and the governance tagging taxonomy applicable to PHA-domain data; and (3) a synthetic, fully de-identified representative dataset (delivered as flat files — e.g., JSON, CSV, or FHIR bundles — reflecting DD Form 3024 Parts A through C2), consistent with the data handling restrictions at Section 7.

No Government-hosted environment is required to pass the Phase 2 CDP Conformance Gate. The Contract Data Package is documentation and data artifacts, not infrastructure access, and is provided so that offerors compete on their ability to conform to a defined interface, rather than on whether they have separately built an equivalent orchestration backbone prior to this AOI. This approach also ensures no offeror gains an evaluation advantage from prior familiarity with a specific Government-hosted platform at the Phase 2 gate; all offerors build to the same published, platform-neutral specification. Connectivity to a Government-hosted environment is evaluated separately, only after Phase 2 is passed, and the Government awards a contract under section 4.9; at the Phase 3 gate described at Section 4.5.4.

Offerors are not required to replicate, replace, or independently stand up the underlying hosting or governance platform to pass Phase 2. That gate is a conformance requirement: offerors must demonstrate that PHA data produced by their capability can be ingested, transformed, and governance-tagged in accordance with the published Contract Data Package, within an environment of the offeror's own choosing.

4.5.2 Minimum Viable Validation Scope

For purposes of the Phase 2 and Phase 3 down-select gates, the data validation requirement is bounded to the PHA data domain — ingestion, transformation, and governance tagging of DD Form 3024 Parts A through C2 data elements in conformance with the Contract Data Package, and connection of that conformant data into the Government-provisioned test environment described at Section 4.5.4. Neither gate requires, and offerors will not be evaluated on, consolidation of the Military Service readiness system data landscape. Enterprise-wide consolidation matures over time under the MOSA-based modularity principles governing the broader PARATUS architecture and is addressed progressively through Phases 4 and 6 below, and through subsequent Areas of Interest.

This bounded scope is intended to keep Phases 2 and 3 achievable within the program's 8-month credibility window (Section 6) for any offeror or team capable of building to the published specification and connecting to the Government-provisioned environment, rather than only those with pre-existing, full-scale DOW/DHA orchestration deployments.

4.5.3 CDP Conformance Validation Method

Conformance to the Contract Data Package is verified as follows during the Phase 2 down-select gate:

- The offeror or team ingests the Government-furnished synthetic dataset into an environment of its own choosing, live, as part of the Phase 2 demonstration.
- The offeror or team demonstrates transformation and governance tagging of that dataset in accordance with the schema, FHIR R4 profile constraints, and tagging taxonomy in the Contract Data Package.
- The offeror or team produces a representative output extract, which the Government validates against the Contract Data Package using a Government-developed conformance check — combining automated schema/structural validation with a manual governance-tag spot-check by the peer review panel.

- Conformance validation is identical in method and rigor for every offeror or team, regardless of prior experience with any specific Government-hosted platform, consistent with the non-scoring of corporate pedigree at Sections 4.6 and 4.8.

The output extract validated under this method is the same extract the offeror or team subsequently transfers into the Government-provisioned test environment at the Phase 3 gate described at Section 4.5.4. Passing Phase 2 is a prerequisite to being evaluated at Phase 3.

4.5.4 Government Test Environment Connection

The Government will provision, in support of the Phase 3 down-select gate described at Section 4.7, a test environment populated with the similar synthetic dataset described at Section 4.5.1. Offerors or teams that have passed Phase 2 may satisfy the Phase 3 gate through either of the following connectivity options:

- Native deployment — a representative instance of the offeror's or team's capability is deployed within an isolated workspace provisioned by the Government for that offeror or team; or
- Remote connection — the offeror or team establishes a secure remote connection from its own environment into the Government-provisioned workspace, consistent with the offeror's proposed hosting approach at Section 4.9.

Under either option, the offeror or team shall demonstrate live data curation and capability workflow demonstrated during Phase 1 within that workspace. The Government will provide sample use cases the offerors can select to demonstrate during this gate to all offerors that pass Phase 2. The Government will confirm workspace provisioning and connectivity options with the platform team in advance of Phase 3 scheduling, and will notify offerors and teams of any environment-specific connection requirements (e.g., network path, credential provisioning, workspace access windows) with reasonable advance notice.

This gate does not require the offeror or team to demonstrate enterprise-scale orchestration, consolidation of legacy data sources, or operational-scale data volumes; it requires only that the offeror's or team's capability can connect to the environment, curate and condition the data, and perform selected uses cases for Government review using all the safeguards and requirements from Phase 2.

4.5.5 Maturation of PHA Capability Across the Six-Phase Structure

Consistent with Section 4.7, this AOI's down-select structure includes two sequential, pass/fail gates specific to data validation, in addition to the Phase 1 PHA capability demonstration: the CDP Conformance Gate (Phase 2) and the Government Test Environment Connection Gate (Phase 3). An offeror or team must pass Phase 2 to be evaluated at Phase 3, consistent with the sequential down-select structure at Section 4.8. PHA capability is evaluated and matured across the full six-phase structure as follows:

- Phase 2 — Offerors demonstrate ingestion, transformation, and governance tagging of PHA-domain data in conformance with the Contract Data Package described at Section 4.5.1, at the minimum viable scope described at Section 4.5.2, verified using the conformance validation method at Section 4.5.3.
- Phase 3 — Offerors connect their Phase 2 conformant output to the Government-provisioned test environment described at Section 4.5.4, via native deployment or

remote connection, and demonstrate a working PHA capability from Phase 1 and Phase 2 with similar synthetic data using a selected or combination of selected Government provided use case(s).

- Phase 4 — The Government issues a revised scenario that may include a clinical change, a revised Contract Data Package element (e.g., a new data element, a revised governance tag, or a modified mapping rule), a change to the Government test environment's connection parameters, or any combination of the three, so that adaptability is evaluated across the clinical capability and both validation gates together.
- Phase 5 — The Service Adoption and Transition Brief shall address the offeror's proposed approach and timeline for scaling both their PHA capability beyond the Phase 2/Phase 3 minimum viable scope toward full enterprise consolidation, and dependencies on Government-side data and governance readiness. As well as how they plan to convince the Military Services to 'buy' and adopt their PHA capability vice continued use of legacy PHA systems, and proposed test and demonstration plans across the DOW.
- Phase 6 — The Service-level field test validates PHA capability at operational scale — schema compliance, governance tagging accuracy, and data latency — using the Government test environment connection already established and gated at Phase 3. Because connectivity is established and passed as its own gate no later than Phase 3, Phase 6 evaluation is not conditioned on environment availability becoming known only at the field-test stage.

4.5.6 Portability and Standards-Based Integration

Consistent with Section 4.9, an offeror's connection approach is evaluated favorably where it relies on standards-based methods (e.g., FHIR R4 resources, open APIs, containerized data pipeline components) rather than methods specific to a single commercial product, so that the Government's ability to select or modify the underlying hosting and governance platform in the future, consistent with MOSA, is preserved regardless of which offeror's PHA capability is selected.

4.6 Team Proposals and Teaming Arrangements

- Offerors may propose as a team — a prime offeror together with one or more teaming partners or subcontractors — to satisfy the combined PHA capability (Section 4.2), CDP conformance (Section 4.5.1–4.5.3), and Government test environment connectivity (Section 4.5.4) requirements of this AOI. No statute, regulation, or provision of this AOI requires that a single entity independently possess every capability described in Sections 4.2 and 4.5.
- Where a team is proposed, evaluation under Sections 4.7 and 4.8 assesses the demonstrated capability of the team collectively, as presented during each phase's live demonstration. Responsibility for specific capability elements (e.g., PHA clinical workflow, CDP conformance, or environment connectivity) may be allocated among team members at the offeror's discretion, provided the team demonstrates the full, integrated capability required by Section 4.2 and the minimum viable validation scope at Section 4.5.2.
- Consistent with Section 4.8, prior corporate experience operating specifically within DOW or DHA data environments is not, by itself, a scored evaluation factor for any team

member at any phase. Evaluation is based on the live demonstrated technical performance of the proposed team's capability against the requirements of this AOI.

- Teaming arrangements shall be identified in the Notice of Intent described at Section 7, including the role of each team member with respect to the requirements of Sections 4.2 and 4.5. Formal teaming agreements or subcontract documentation need not be finalized prior to the Phase 1 demonstration, but the Government must be able to identify, for evaluation purposes, which entity is responsible for which demonstrated capability.
- A team's composition may evolve between phases — for example, addition of a connectivity-focused teaming partner between Phase 2 and Phase 3 — provided the Government is notified in advance and the change does not constitute a material substitution of the capability evaluated at the prior phase.

4.7 Phased Evaluation Approach

This AOI is executed in six-phases. Phase 1 and 2 will allow the Government to evaluate Offerors capabilities against the requirements outlined in Section 4.2 through 4.5.1. At the end of Phase 2, the Government may award agreements under section 4.9 to offerors to continue refining their capabilities against Phases 3 through 6. At the completion of Phase 6, the Government intends to award a multiple year agreement to an offeror who will refine, deploy and sustain their capability for production. There is no preliminary written solution brief; Phase 1 is the full capability demonstration, and advancement through each subsequent phase and the award of any agreement under section 4.9 is contingent on successful performance in the phase before it.

Phase 1 — Full Capability Demonstration

Phase 1 is conducted as a single live event but is evaluated sequentially across three distinct elements (1.a, 1.b, and 1.c) to ensure offerors possess both the strategic vision for modernization and the technical baseline to execute it. There is no separate written solution-brief stage preceding this demonstration.

- Phase 1.a — Strategic Vision & Complexity Understanding (Qualitative Gate): Offerors must first present their strategic roadmap and vision for a "post-forms" PHA, addressing the complexities and requirements outlined in Section 4.3. This is a primary qualitative down-select discriminator to ensure offerors understand the department wide data curation effort required to necessitate PARATUS as the flagship system for medical readiness, unencumbered by current policy paradigms and are not constrained by current policy paradigms.
- Phase 1.b — Full Capability Demonstration: Offerors, or teams under Section 4.6, demonstrate, in full and without abridgment, the current capability described in Section 4.2 — Parts A, B, C1, and C2. Conceptual, roadmap, or planned-but-unbuilt capability does not satisfy Phase 1.b. Offerors unable to demonstrate a functioning, end-to-end baseline capability will not advance.
- Phase 1.c — Hosting Environment Flexibility: Offerors shall describe the hosting environment(s) their proposed capability currently operates within, addressing the portability and RMF requirements described in Section 4.9.

Note: The Government reserves the right to conclude a Phase 1 presentation early or decline to evaluate Phase 1.b/1.c, if an offeror's Phase 1.a presentation clearly fails to demonstrate an understanding of the modernization complexities or relies solely on a traditional forms-based paradigm.

Phase 2 — CDP Conformance Gate

Offerors or teams who pass Phase 1 ingest the Government-furnished synthetic dataset (Section 4.5.1) into an environment of their own choosing and demonstrate transformation and governance tagging in conformance with the Contract Data Package, verified using the method at Section 4.5.3. No access to a Government-hosted environment is required to pass this gate. This gate is scored pass/fail: an offeror or team that cannot produce a conformant output extract does not advance to Phase 3, regardless of Phase 1 performance.

Phase 3 — Government Test Environment Connection Gate

Offerors or teams who pass Phase 2, may receive an agreement under section 4.9 to continue maturing their capability and compete for an award by successful completion of remaining phases starting at Phase 3 by connecting to a Government-provisioned test environment described at Section 4.5.4, using either native deployment or a secure remote connection, and demonstrate a working PHA capability from Phase 1 and Phase 2 with similar synthetic data using a selected or combination of selected Government provided use case(s) within the Government-provisioned workspace. This gate is scored pass/fail: an offeror or team that cannot establish, demonstrate this connection, and complete a use case(s) does not advance to Phase 4, regardless of Phase 1 or Phase 2 performance. Once a prototype OT has been awarded under Section 4.9, this phase is administered as a milestone under that agreement, subject to the Government's off-ramp rights, rather than as a competitive down-select gate.

Phase 4 — Adaptability Scenario Evaluation

Offerors or teams who pass Phase 3 are presented with a Government-defined scenario representing an emerging requirement or a change to the PHA process, the Contract Data Package, or the Government test environment connection — for example, a new DD Form 3024 data element, a revised screening question set, an updated risk-scoring criterion, a change to IMR category logic, a revised Contract Data Package element, or a change to the Government workspace's connection parameters. Offerors must implement the change in their capability and demonstrate the result within a Government-specified turnaround window. Evaluation in this phase focuses on the speed, completeness, and quality of adaptation — across the clinical capability, CDP conformance, and Government test environment connectivity, as applicable to the scenario presented. An offeror's or team's ability to reconfigure or extend its capability quickly, without protracted engineering effort or vendor lock-in to a lengthy change-request cycle, is the central discriminator of this phase. Once a prototype OT has been awarded under Section 4.9, this phase is administered as a milestone under that agreement, subject to the Government's off-ramp rights, rather than as a competitive down-select gate.

Phase 5 — Service Adoption and Transition Brief

Offerors or teams who pass Phase 4 present a brief addressing how their proposed PHA capability and orchestration integration would be marketed to, and adopted by, the Military Services as a replacement for current Military Service readiness system variants currently in use for PHA-related medical readiness tracking. The brief must address, at minimum: the

offeror's proposed migration and data-transition approach from legacy systems; anticipated training and change-management requirements for Service members, reviewers, providers, and commanders; the offeror's value proposition relative to the capabilities and limitations of current Military service system variants; and anticipated points of Service-level resistance or adoption risk, with mitigation approaches. The Government will issue briefing instructions to offerors who pass Phase 4 with the notification of Phase 4 fulfillment. Once a prototype OT has been awarded under Section 4.9, this phase is administered as a milestone under that agreement, subject to the Government's off-ramp rights, rather than as a competitive down-select gate.

Phase 6 — Service-Level Field Test Review

Offerors or teams who pass Phase 5 proceed to operational field testing of their capability with one or more Military Services, under conditions and a duration specified by the Government and bounded by the schedule at Section 6. Phase 6 evaluation consists of Government review of the results of that field testing — including completion rates, user feedback from Service Members, reviewers, and providers, system performance and reliability, validation performance at operational scale using the Government test environment connection established at Phase 3, and any safety or clinical-accountability findings related to the human-in-the-loop safeguard in Section 4.4. Successful completion of Phase 6 is the final technical prerequisite to a request for full pricing and consideration for award. Once a prototype OT has been awarded under Section 4.9, this phase is administered as a milestone under that agreement, subject to the Government's off-ramp rights, rather than as a competitive down-select gate.

4.8 Scoring Approach

At Phases 1, 4, 5, and 6, the offeror's approach is assessed by a peer review panel of scientific, technical, clinical, or other subject-matter experts, using an adjectival rating scale (e.g., Outstanding / Good / Acceptable / Marginal / Unacceptable).

Outstanding	Proposal demonstrates an exceptional approach and understanding of the requirements and risk of unsuccessful performance is low.
Good	Proposal indicates a thorough approach and understanding of the requirements and risk of unsuccessful performance is low to moderate.
Acceptable	Proposal meets requirements and indicates an adequate approach and understanding of the requirements, and risk of unsuccessful performance is no worse than moderate.
Marginal	Proposal has not demonstrated an adequate approach and understanding of the requirements, and/or risk of unsuccessful performance is high.
Unacceptable	Proposal does not meet requirements of the solicitation and is unawardable, and/or risk of performance is unacceptably high.

Phases 2 and 3 — the CDP Conformance Gate and the Government Test Environment Connection Gate — are scored pass/fail, not adjectivally: an offeror or team either produces a conformant output extract and connects it to the Government-provisioned environment as

described at Sections 4.5.3 and 4.5.4, or does not. This mirrors the pass/fail treatment already given to the human-in-the-loop safeguard below, and reflects that these two capabilities are threshold requirements for continued participation rather than a basis for comparative technical scoring.

Adjectival Rating	Description
Pass	Based on the offeror's approach, the Government has a reasonable expectation that the offeror will successfully perform the required effort.
Fail	Based on the offeror's approach, the Government does not have a reasonable expectation that the offeror will be able to successfully perform the required effort.

Compliance with the human-in-the-loop safeguard in Sections 3 and 4.4 is a pass/fail gating criterion at every phase, not a graded one: an offeror's or team's solution that does not preserve final clinical or diagnostic authority with a licensed Health Care Provider, or Independent Duty Enlisted Provider with any necessary oversight, will be eliminated from further consideration at whichever phase the deficiency is identified, regardless of technical sophistication elsewhere in its capability.

Evaluation of the data validation requirement at Phases 2, 3, 4, 5, and 6 is based on demonstrated technical conformance with the Contract Data Package and connectivity with the Government-provisioned test environment, per Sections 4.5.1 through 4.5.5. Consistent with Section 4.6, prior corporate experience operating specifically within DOW or DHA data environments is not, by itself, a scored evaluation factor for any offeror or team member at any phase; an offeror or team that demonstrates the required validation conformance and connectivity live, without prior DOW/DHA-specific deployment history, is evaluated on equal footing with one that has such history.

Advancement is sequential: an offeror or team must pass Phase 1 to be evaluated in Phase 2, must pass Phase 2, and awarded an agreement identified in section 4.9 to be evaluated in Phase 3, must pass Phase 3 to be evaluated in Phase 4, must pass Phase 4 to be evaluated in Phase 5, and must pass Phase 5 to proceed to Phase 6. The Government may, at its discretion, decline to advance an otherwise-passing offeror or team if a more favorably evaluated competing offeror's or team's capability would better close the capability gap described in Section 2, consistent with the CSO's competitive, non-ranked peer-review structure.

Strategic Vision vs. Baseline Capability

Evaluation of the Strategic Vision (Phase 1.a) and Additional Capabilities is tightly coupled with the baseline capability demonstration (Phase 1.b). While offerors are required to present forward-looking capabilities and strategic vision to pass the qualitative Phase 1.a gate, a strong visionary presentation will never compensate for a failure to meet the minimum baseline software requirements. An offeror must successfully demonstrate a working baseline capability (Section 4.2) during Phase 1.b to advance. Promises of future innovation under Section 4.3 are never treated as compensating for a failure to satisfy the baseline requirements of Sections 4.2, 4.4, 4.5.3, or 4.5.4.

4.9 OT Award Following Phase 2; Post-Award Milestone Structure

4.9.1 Basis and Discretion to Award

Following an offeror's or team's successful completion of Phase 2 (CDP Conformance Gate), the Government may, in its sole discretion, award a prototype Other Transaction agreement under 10 U.S.C. § 4022 to that offeror or team.

4.9.2 Scope of the OT Agreement

An OT agreement awarded under this section covers continued performance of Phases 3 through 6 as described at Section 4.7. Award under this section is not a re-competition of Phase 1 or Phase 2; it extends only to the offeror or team that passed those phases under this AOI.

4.9.3 Reserve Offerors

At the time of an OT award under this section, the Government may designate one or more other offerors or teams who have passed Phase 2 as reserve offerors. A reserve offeror or team so designated:

- Remains eligible for OT award under this section, without repeating Phase 1 or Phase 2, for 180 days from the date its Phase 2 result was confirmed, unless extended in writing by the Government;
- Shall have its technical and pricing submissions re-verified before any award under this provision if more than 90 days have elapsed since its Phase 2 result was confirmed; and
- May be awarded an OT agreement under this section if the offeror or team initially awarded under Section 4.9.1 fails a subsequent milestone under Section 4.9.4, fails to execute the OT agreement, or is subsequently found non-responsible.

Designation as a reserve offeror or team confers no right to award and does not entitle the offeror or team to notice of, or participation in, the Government's administration of another offeror's or team's OT agreement.

4.9.4 Post-Award Competitive Down Select Structure

For an offeror or team awarded an OT agreement under this section, Phases 3 through 6 at Section 4.7 are administered as competitive down select gates prescribed in section 4.7 under that agreement.. The technical criteria described for each phase at Section 4.7 continue to apply, and a failure to meet a phase terminates the OT prototype agreement.

4.9.5 Pricing

For an offeror or team awarded an OT agreement under this section, provisional, not-to-exceed pricing is established at the time of award, in lieu of the post-Phase 6 price reasonableness review described at Section 5. The Post-Phase 6 pricing review at Section 5 applies only to an offeror or team that continues under this CSO as a standard down-select through Phase 6 without an OT award under this section.

4.9.6 Follow-On Production Authority

Follow-on production for an offeror or team awarded a prototype OT agreement under this section is addressed at Section 1, which governs the Government's election between extending or converting that agreement under 10 U.S.C. § 4022(f) and awarding a new contract under another appropriate authority. This section does not itself restrict that election to either authority.

5.0 Hosting Environment Flexibility

The Government's minimum hosting requirement is operation within a DOW Impact Level 5 (IL5) environment with the ability to integrate in an IL6 CDS, consistent with Section 3. The Government does not mandate a specific cloud service provider, hosting platform, or data architecture as a condition of this CSO, and does not wish to narrow competition to offerors whose solutions are built specifically around any single Government-hosted environment.

Offerors shall describe the hosting environment(s) their proposed capability currently operates within or can be readily adapted to, and shall address the following during the Phase 1 capability demonstration:

- **Portability** — the degree to which the proposed capability relies on portable, standards-based architecture (e.g., containerized services, standard APIs) versus proprietary services specific to a single hosting environment, such that the capability could be migrated to a different DOW-authorized IL5 w/ IL6 CDS environment without a full re-architecture.
- **Demonstrated or Readily Achievable Multi-Environment Operation** — evidence that the capability operates today, or could be adapted with reasonable effort, within more than one DOW-authorized IL5 w/ IL6 CDS hosting environment (e.g., War Data Platform/Databricks, a major cloud service provider's DOW-authorized Government environment, or an on-premises DOW enclave).
- **Authorization to Operate (ATO) Portability** — a description of how the capability's security accreditation package, including its data integration layer, would transfer, or be substantially reusable, if hosted in a different DOW-authorized IL5 w/ IL6 CDS environment, rather than requiring a full new ATO effort per environment.
- **MHS Data Integration Independent of Hosting Location** — confirmation that the data integration requirements in Sections 3, 4.4, and 4.5 (e.g., MHS Genesis FHIR R4 integration, Contract Data Package conformance, Government test environment connectivity) can be satisfied regardless of which DOW-authorized IL5 w/ IL6 CDS environment ultimately hosts the capability.
- **Choice of Connectivity Option at Phase 3** — an offeror's election between native deployment and remote connection at Section 4.5.4 is not itself scored; both options satisfy the Phase 3 gate equally, and the offeror's election should be consistent with the portability and multi-environment posture described above.

Hosting environment flexibility is evaluated as a favorable discriminator during Phase 1, consistent with Section 5, but is not a gating criterion: an offeror whose capability is hosting-environment-specific today, but otherwise fully satisfies Sections 4.2, 4.3, 4.4, and 4.5, remains eligible to advance. The Government's intent is to preserve its own future flexibility to host the capability where it makes the most operational and cost sense over the life of the program,

without requiring offerors to abandon an existing, proven hosting approach as a condition of competing.

6. Evaluation Factors

The table below reflects the AOI's six-phase structure:

Phase	Factor	Basis of Evaluation
Phase 1.a	Strategic Vision & Complexity Understanding	Adjectival rating. Initial qualitative gate. Evaluates the offeror's approach to a "post-forms" continuous assessment and profiling environment and their understanding of PARATUS integration complexities per Section 4.3.
Phase 1.b	Technical — PHA Capability (Parts A–C2)	Full, end-to-end live demonstration per Sections 4.2 and 4.4; the primary clinical-capability technical discriminator. May be satisfied by a team per Section 4.6.
Phase 1.c	Hosting Environment Flexibility	Favorable (non-gating) discriminator; portability, multi-environment operation, and ATO portability across DOW-authorized IL5 w/ IL6 CDS environments per Section 4.9.
Phase 2	Technical — CDP Conformance Gate	Pass/Fail. Live demonstration of ingestion, transformation, and governance tagging in conformance with the Government-furnished Contract Data Package, verified via the conformance validation method at Section 4.5.3, at the minimum viable scope defined at Section 4.5.2. No Government-hosted environment required. Prior DOW/DHA-specific corporate experience or environment access is not scored, per Sections 4.6 and 4.8.
Phase 3*	Technical — Government Test Environment Connection Gate	Pass/Fail. Live connection (native deployment or remote connection) of the Phase 1 PHA capability to the Government-provisioned test environment per Section 4.5.4; live demonstration of selected use case(s) in the provisioned test environment.
Phase 4	Adaptability	Adjectival rating. Speed and quality of implementing a Government-defined emerging requirement or process change, including a revised Contract Data Package element and/or Government test environment connection parameter per Section 4.5.5.
Phase 5	Service Adoption Strategy	Adjectival rating. Feasibility of migration, training, and change-management approach versus current Military service readiness system variants, including the plan for scaling CDP conformance and Government test environment connectivity per Section 4.5.5.
Phase 6	Field Test Performance	Adjectival rating. Government review of Service-level operational field test results, including reliability, orchestration performance at operational scale via the Phase 3 connection, and safeguard compliance.
All Phases –	Human-in-the-Loop Safeguard Compliance	Pass/Fail gating at every phase; preservation of licensed Health Care Provider authority over clinical/diagnostic determinations.
Post-Phase 6	Price	Evaluated for reasonableness following successful Phase 6 completion, not a down-select factor at any earlier phase. For an offeror or team awarded a prototype OT agreement under Section 4.9, this row does not apply; pricing is instead established as provisional, not-to-exceed pricing at the time of OT award, per Section 4.9.5.

*Note: The Phase 3 through Phase 6 rows above describe the basis of evaluation for an offeror or team continuing under this CSO as a standard down-select. For an offeror or team awarded a prototype OT agreement under Section 4.9 following Phase 2, these same technical criteria apply, but function as milestone-acceptance criteria under that agreement rather than down-select scoring factors, per Sections 4.8 and 4.9.4.

Technical merit — spanning PHA clinical capability, CDP conformance, and Government test environment connectivity — is the primary evaluation factor throughout Phases 1–6, subject to the human-in-the-loop safeguard as a gating criterion at every phase, and subject to the Phase 2 and Phase 3 pass/fail gates described at Section 4.8. Price is considered only after Phase 6 completion, and only to the extent necessary to determine fair and reasonable cost for an award outlined in section 4.9

7. Program Timeline and Period for Acceptance of Proposals

Requests to participate in Phase 1 demonstrations will be accepted on a rolling basis for 7 days from the date of CSO publication on SAM.gov. The Government reserves the right to schedule and conduct Phase 1 demonstrations, and subsequent phase evaluations, at periodic intervals during the open period rather than waiting for the period to close, consistent with standard CSO practice.

This AOI is structured to reach a down-select decision and field-validated capability within an 8-month credibility window from CSO publication. The schedule below bounds each of the six phases at Section 4.7 to fit within that window; durations may be compressed further at the Government's discretion where offerors or teams are ready to proceed sooner.

Milestone	Projected Target Window	Scope
CSO Publication / Notice of Intent	Days 1–7	AOI publishes on SAM.gov; offerors and teams submit Notice of Intent per Section 7, including teaming roles per Section 4.6.
Phase 1 — Full Capability Demonstration	Days 8–28 (~3 weeks)	Live Strategic Vision (4.3) and PHA Capability (4.2) demonstrations. Data orchestration is not evaluated at this phase.
Phase 2 — CDP Conformance Gate	Days 29–42 (~2 weeks)	Offerors ingest the synthetic dataset and demonstrate conformance against the published Contract Data Package (Section 4.5.1), verified via the conformance check at Section 4.5.3; no Government-hosted environment required. Pass/fail.
OT Award Decision Point (if elected)	Days 43–50 (~1 week)	The Government determines whether to award a prototype OT agreement under Section 4.9 to the offeror or team that passed Phase 2, in lieu of continuing that offeror's or team's participation as a standard CSO down-select through Phase 6.
Phase 3 — Government Test Environment Connection Gate	Days 51–63 (~2 weeks)	Offerors connect their Phase 1 PHA capability to a Government-provisioned test environment (Section 4.5.4) via native deployment or remote connection, and provide live demonstrations of Government provided use case(s). Pass/fail.
Phase 4 — Adaptability Scenario	Days 64–91 (~4 weeks)	Government issues scenario including a clinical change, a revised Contract Data Package element, and/or a Government test environment connection parameter change per Section 4.5.5; offerors/teams implement and demonstrate within the window.
Phase 5 — Service Adoption and Transition Brief	Days 92–98 (~1 week)	Migration, training, change-management, and orchestration-scaling brief per Sections 4.7 and 4.5.5.
Phase 6 — Service-Level Field Test	Days 99–225 (~4.2 months)	Operational field test with one or more Military Services; orchestration validated at operational scale using the Government test environment connection established at Phase 3.

Milestone	Projected Target Window	Scope
Down-Select Decision / Pricing Review	Days 226–240	Government review of Phase 6 results; price reasonableness review per Section 5; down-select decision within the 8-month window.

Where the Government elects to award a prototype OT agreement under Section 4.9, the Phase 3 through Phase 6 windows above continue to bound the corresponding post-award milestones under that agreement, per Section 4.9.4, but are administered as contract milestones rather than CSO down-select phases from that point forward. Where no OT agreement is awarded under Section 4.9, the timeline above applies without modification and the down-select decision at the close of Phase 6 proceeds under Section 4.8.

Given the six-phase structure, offerors and teams should anticipate a multi-month evaluation timeline from Phase 1 demonstration through Phase 6 field test review, bounded by the approximately 240-day (8-month) window above. The Government may compress Phase 6 field test duration for an offeror or team that demonstrates readiness ahead of the target window, but will not extend the overall window beyond 8 months from CSO publication without a corresponding amendment to this AOI.

8. Submission Instructions

Because Phase 1 is the full capability demonstration rather than a written solution brief, offerors shall submit a brief Notice of Intent to Demonstrate, not to exceed two pages via email to Elizabeth.m.ruckwardt.civ@health.mil. The Notice of Intent to Demonstrate shall identify the offeror's (or team's) point of contact, a summary description of the capability to be demonstrated, any teaming arrangement and the role of each team member per Section 4.6, and any environment or access requirements needed to support the Phase 1 demonstration and the Phase 3 Government test environment connection. Technical evaluation does not occur based on the Notice of Intent to Demonstrate; it occurs solely on the basis of the live demonstration and subsequent phases described in Section 4.7. The Notice of Intent to Demonstrate should identify whether the proposed capability is currently operational, and if so, in what environment and at what DOW, Federal, or commercial healthcare customer(s), to the extent releasable; identification of such prior deployments is for informational purposes only and, consistent with Sections 4.8 and 4.9, is not a scored evaluation factor.

Notice of Intent to Demonstrate submissions, and all data used in or generated during Phase 1 and any subsequent phase, shall not include any actual personally identifiable information (PII) or protected health information (PHI). Any sample data, screenshots, or demonstration data must be synthetic or fully de-identified throughout all phases of this CSO.

Offerors should self-identify nontraditional defense contractor (NDC) status, if applicable, per the definition at DFARS 212.70 and 10 U.S.C. § 4022.

Questions regarding this AOI shall be directed to Elizabeth.m.ruckwardt.civ@health.mil. This AOI does not commit the Government to award and does not entitle any offeror to reimbursement of proposal preparation costs.